MACQUARIE UNIVERSITY Health

MACQUARIE MEDICAL IMAGING

MMI@THE HOSPITAL
Macquarie University Hospital
Ground Floor, 2 Technology Place
Macquarie University NSW 2109
ABN: 46 141 203 125
MQ Health Pty Ltd

MMI@MACQUARIE CENTRE
Shop 1046 (near the ice rink) Macquarie Centre
Cnr Herring and Waterloo Roads
North Ryde NSW 2113
ABN: 46 141 203 125
MQ Health Pty Ltd

P: 02 9430 1100 | F: 02 9430 1199 | E: mmi.enquires@mqhealth.org.au | www.mqmi.com.au

Patient Name:	WANG, DONGPING	**Accession Number:**	3362266-MR
Patient ID:	MQ152436	**Examination Date:**	October 26, 2026 11:25
Gender:	Male	**Report Status:**	Final
Date of Birth:	January 7, 1957	**Modality:**	MR
Referring Physician:	LIN, CHARLIE		

Report for Magnetic Resonance

MRI RIGHT KNEE

HISTORY: Locking episodes, ?meniscal injury.

TECHNIQUE: Axial, coronal and sagittal PD fat-sat and PD sequences.

FINDINGS:

ACL and PCL are intact.

Medial meniscus is intact. There is no displaced meniscal flap or extrusion. There is full-thickness cartilage loss in the inner weightbearing surface of the medial femoral condyle. Intact articular cartilage on the medial tibial plateau. MCL is intact.

Lateral meniscus is intact. There is a chronic full-thickness cartilage defect in the lateral tibial plateau inner posterior aspect, measuring 12 x 14mm, with some evidence of remodelling. Mild degeneration of the proximal LCL attachment. No acute tear or sprain. Biceps femoris and popliteus tendons appear normal. Minimal oedema adjacent to the distal ITB.

Full-thickness cartilage loss in the lateral patellar facet superiorly with underlying subchondral cystic change and oedema. Moderate-grade chondral wear in the femoral trochlea centrally with full-thickness chondral fissuring in the medial femoral trochlea inferiorly.

Hoffa's fat pad appears normal.

There is tendinosis of the proximal patellar tendon and enthesopathy of the distal quadriceps and proximal patellar tendon.

Small knee joint effusion. No intra-articular loose body is seen. No Baker’s cyst.

Neurovascular structures appear normal.

CONCLUSION:

No meniscal tear.

Areas of full-thickness cartilage loss in the medial femoral condyle, lateral tibial plateau, patella and femoral trochlea.

Moderate proximal patellar tendinosis.

Small knee joint effusion.

Radiologist: Dr S. McDonald